5.1 Stroke (Ischemic and Hemorrhagic)*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Family report sudden right-sided weakness and slurred speech 2 hours ago **Objective** • Right hemiparesis, power 2/5 • Facial droop on one side • Cannot turn in bed unaided	**ACTUAL** Impaired physical mobility related to neuromuscular impairment as evidenced by hemiparesis and inability to turn independently.	• Joint range preserved without contracture; assisted mobilization under way within 1 week.	• Assess neurological status and GCS as ordered, often hourly in the acute phase. • Keep the weak limbs in neutral alignment and support the affected shoulder. • Turn him every 2 hours. • Perform passive ROM three times a day. • Start early rehabilitation with the physiotherapist.	• Considered met when there are no contractures and mobility is improving.
Subjective • Tries to speak but words are unclear **Objective** • Expressive aphasia • Frustrated	**ACTUAL** Impaired verbal communication related to cerebral injury as evidenced by inability to articulate words.	• Makes basic needs known by other means within 3 days.	• Speak slowly, in short, simple sentences. • Give him time to answer and do not finish his sentences. • Use picture boards and gestures. • Refer to the speech and language therapist.	• Goal reached once basic needs are communicated.
Subjective • Coughs when drinking **Objective** • Facial weakness • Weak gag reflex	**RISK** Risk for aspiration as evidenced by risk factors: impaired swallowing and reduced level of consciousness.	• No aspiration.	• Nil by mouth until the swallow screen is done. • Sit him upright for meals and for 30 minutes afterward. • Give thickened fluids as recommended. • Keep suction ready.	• Met while breath sounds stay clear.

* **References for this care plan:** [17], [35], [37], [111], [124]. Full entries are listed in the References section of the CPMN book.